The salt isn't *the whole story.*

A home blood pressure guide from a night hospitalist

For anyone who has cut back on salt, is doing their best, and is still seeing numbers that won't settle down.

> *"But doctor, I stopped adding salt."*
>
> I hear some version of this sentence often on night shift, usually from someone who has genuinely tried. They put the salt shaker away. They feel fine. And yet the pressure is high enough that they've ended up in a hospital bed at 3 a.m. Their effort was real. The plan just wasn't finished yet. This guide is about finishing it, calmly, before it becomes an emergency.

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THE ONE LESSON

Cutting salt is a good start. Your home readings are what tell you whether the plan is working.

PART 1

Why your pressure can *stay high*

Blood pressure rarely has one cause. Think of it as a pattern with several threads. Pulling one thread helps, and the readings show you which threads are still tight.

01 Cutting salt helps, but it may not be enough

People respond to sodium differently. For some, cutting back lowers pressure a lot. For others, only a little. Keep the change, and then check whether your readings actually improve.

02 The salt shaker isn't where most sodium lives

Most of the sodium we eat is already inside food before it reaches the table: bread, deli meats, sauces, canned soups, restaurant and takeout meals. Many of these don't even taste salty. (The next two pages make this easy.)

03 Your whole eating pattern matters

DASH-style eating is built around vegetables, fruit, whole grains, beans, nuts and lower-fat dairy. It's about what you add to your plate, not only what you take away.

04 Other habits count too

Regular movement, weight management when it's appropriate for you, and drinking less alcohol each help. They work alongside sodium reduction, not instead of it.

05 Something else may be contributing

Sleep apnea, kidney problems, and some medicines (like ibuprofen, naproxen and decongestants) can push pressure up. So can some supplements. Page 5 helps you ask about these.

06 Medication isn't a failure

Many people need medicine alongside lifestyle changes, and sometimes more than one. That isn't a sign you did something wrong. It's a sign your body needs more support.

07 Feeling fine doesn't tell you your pressure

High blood pressure is usually silent. The only way to know your numbers is to measure them, which is why the measuring steps and home log in this guide matter so much.

PART 2 · THE HIDDEN SODIUM SPOTTER

Salt is a sneaky hider!

Salt doesn't just live in the salt shaker. It loves to hide inside food, even food that doesn't taste salty. Let's find where it's hiding!

Written simply on purpose. Read it with your kids or grandkids, or cut the cards out and stick them on the fridge.

START HERE



MEET

Sneaky Salt

Grown-ups call salt "sodium." A little is okay. Too much makes the heart work extra hard. Most of it is hiding in food we buy, not the shaker!

CARD 1



HIDING SPOT

Bread & rolls

Bread doesn't taste salty. But we eat it a LOT: toast, sandwiches, buns. Little bits of salt in each slice add up fast!

CARD 2



HIDING SPOT

Lunch meats

Ham, turkey slices, hot dogs, bacon. Salt is added to help them last longer, so there's a lot of it inside.

CARD 3



HIDING SPOT

Pizza & sandwiches

Bread + cheese + meat + sauce. Each one brings some salt, so stacking them together makes a big salt tower!

CARD 4



HIDING SPOT

Canned soup

Soup feels cozy and healthy, but one can may hold a big scoop of salt. Look for cans that say "low sodium."

CARD 5



HIDING SPOT

Sauces & dressings

Ketchup, soy sauce, salad dressing, seasoning packets. Little squirts, but BIG salt for their size!

CARD 6



HIDING SPOT

Restaurant & takeout

Food made away from home often has extra salt to make it taste strong. Home cooking lets YOU be the boss of the salt.

CARD 7



HIDING SPOT

Salty snacks

Chips, crackers, pretzels and salted nuts. These ones taste salty, so they're the easiest hiders to catch!

PART 3 · BE A LABEL DETECTIVE 🔍

Find the magic number!

On the back of food packages there's a box called **Nutrition Facts**. Find the word **Sodium**, then look at the number with the **%** sign next to it.

Nutrition Facts

Serving size	**1 cup**
Calories	**150**
	% Daily Value*
Total Fat 4g	5%
Sodium 690mg	**30%**
Total Carb 20g	7%
Protein 6g	

Example label for teaching only

Detective steps:

1. Find **Sodium**.
2. Look at the **%** number. 30% here? That's HIGH!
3. Check the **serving size**. If you eat 2 cups, you get double the salt: 60%!
4. Compare two packages. Pick the smaller number. 🏆

5%
or less = LOW
Little salt. Great pick! 👍

6–19%
= IN BETWEEN
Okay sometimes. Keep an eye on it. 👀

20%
or more = HIGH
Lots of salt! Have it less often. ✋

Swap game: trade the hider for a helper!

🥓 → 🍗
Deli meat → chicken you cooked yourself

🥫 → 🍲
Regular canned soup → low-sodium or homemade

🧂 → 🍋
Extra salt → lemon, garlic, pepper & herbs

🍟 → 🥕
Chips → crunchy veggies or unsalted nuts

🥫 → 💧
Canned beans & veggies → give them a rinse

🍔 → 🏠
Takeout every night → cook at home more often

For most grown-ups, the salt limit for the **WHOLE day** is about **1 teaspoon** (2,300 mg of sodium). Many people with high blood pressure are told to aim even lower.

For adults: the American Heart Association suggests an ideal of 1,500 mg a day for many people, especially those with high blood pressure. Ask your clinician what target fits you.

PART 4

The bigger pattern, *and what else to ask about*

Sodium is one thread. Here are the others worth noticing, and the clues worth bringing to your clinician.

A DASH-style plate

Think about what to add more of, not only what to cut.

- [ ] Half the plate vegetables and fruit, most days
- [ ] Whole grains: oats, brown rice, whole-wheat bread
- [ ] Beans, lentils, nuts and seeds a few times a week
- [ ] Lower-fat dairy, like milk or yogurt
- [ ] Fish, poultry and plant proteins more often than red or processed meat
- [ ] Fewer sugary drinks and sweets

Habits that work alongside it

- [ ] Regular movement you can keep up, like brisk walking
- [ ] Weight management, when your clinician says it's right for you
- [ ] Less alcohol, or none
- [ ] Rest and sleep, which matter more than people expect

Clues worth asking about

Bring these up at your next visit. They're conversation starters, not diagnoses.

- [ ] **Sleep apnea.** Loud snoring, gasping at night, or waking up tired after a full night's sleep.
- [ ] **Kidney health.** Ask whether your kidney function and urine have been checked.
- [ ] **Pain relievers.** Regular ibuprofen or naproxen can raise pressure.
- [ ] **Cold medicines.** Decongestants can raise pressure.
- [ ] **Supplements.** Bring every bottle, including herbal and “natural” ones, so they can be reviewed.

ONE IMPORTANT NOTE

Don't stop a prescription on your own, even after cutting salt or getting one normal reading. Talk to your clinician first. That normal reading may be the medicine working.

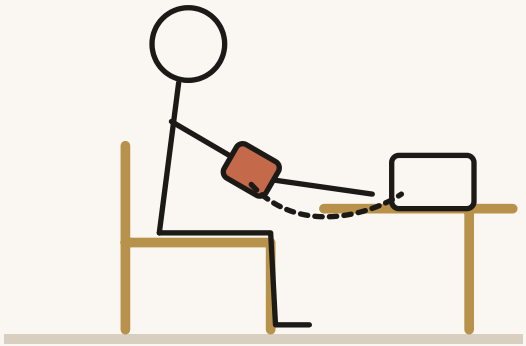
PART 5

How to measure it *the right way*

A rushed reading can look higher than your real pressure. A careful one gives you a number you and your clinician can trust. Keep this page near your cuff.

- [ ] **Use a validated upper-arm cuff** that fits your arm. Wrist cuffs and smartwatches are less reliable.
- [ ] **For 30 minutes before:** no caffeine, exercise or smoking. Use the bathroom first.
- [ ] **Sit and rest quietly for 5 minutes** before you start. No phone, no talking.
- [ ] **Back supported, feet flat,** legs uncrossed.
- [ ] **Arm supported at heart level,** cuff on bare skin, not over a sleeve.
- [ ] **Take 2 readings, 1 minute apart,** and write both down.
- [ ] **Same times each day,** morning and evening, so readings can be compared.
- [ ] **Bring your cuff to an appointment** once so it can be checked against the office reading.

The 5-minute setup



Back supported · feet flat · arm resting at heart level · cuff on bare upper arm

PART 6 · WHEN A NUMBER NEEDS ACTION

Above 180 top number OR above 120 bottom number, *with* symptoms

Chest pain, shortness of breath, back pain, weakness or numbness, vision changes, or trouble speaking.

Call 911. Don't wait to see if it improves.

The same numbers, *without* symptoms

Sit quietly and repeat the reading after at least 1 minute.

If it's still that high, contact your clinician promptly (the same day). Don't change your medicines on your own.

PART 7

Your 7-day *home log*

Write down both readings each time, top number / bottom number. One reading is a snapshot. A week of readings shows the pattern your clinician needs.

Name: _____ Arm used: ☐ Left ☐ Right Week of: _____

DAY	MORNING			EVENING			NOTES
	TIME	READING 1	READING 2	TIME	READING 1	READING 2	SYMPTOMS · MISSED MEDS · SALTY MEAL · POOR SLEEP
Day 1		/	/		/	/	
Day 2		/	/		/	/	
Day 3		/	/		/	/	
Day 4		/	/		/	/	
Day 5		/	/		/	/	
Day 6		/	/		/	/	
Day 7		/	/		/	/	

Morning
Before breakfast and before your medicines, after using the bathroom.

Evening
Before dinner or before bed, at about the same time each day.

Medicines
List what you take and when: _____
_____

Bring it
Take this page (and your cuff) to your next appointment.

PART 8

Your appointment page

Hand over your log and start with the first question. The rest help you leave with a clear plan.

1 “Here are my home readings. Is my current plan enough?”
The single most useful sentence you can bring.

2 “What blood pressure number are we aiming for, for me?”
Targets can differ from person to person.

3 “Could any of my medicines or supplements be raising my pressure?”
Bring the bottles or a photo of each label.

4 “Should I be checked for sleep apnea or kidney problems?”
Especially if you snore, wake up tired, or haven't had labs recently.

5 “How often should I check at home, and what numbers mean I should call you?”
Write the answer below so it's there when you need it.

Your readings aren't a grade. *They're information*, and information is something you can act on.

If you've cut back on salt, that effort counts. Medicine isn't a failure. A higher number isn't a judgment. Each careful reading moves you closer to a plan that actually fits your body.

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General education, not personal medical advice. This guide doesn't replace care from your own clinician, who knows your history. In an emergency, call 911. Sources: 2025 AHA/ACC High Blood Pressure Guideline; American Heart Association guidance on home monitoring, sodium and the “Salty Six”; U.S. FDA guidance on reading %Daily Value on the Nutrition Facts label.